Osimertinib monotherapy

Which treatment this guide covers This guide covers NCCP 00353 osimertinib on its own: T790M-positive advanced disease, first-line activating-EGFR advanced disease, or completely resected stage IB–IIIA Ex19del/L858R adjuvant treatment. The newer licensed post-chemoradiation setting and osimertinib with platinum/pemetrexed need their own applicable plan; their study results and support medicines are not added to this monotherapy suite.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Osimertinib alone · daily oral treatment

Active medicines: osimertinib

Emetogenic risk: Minimal to low (oral)

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Agree an EGFR-inhibitor skin-care plan from the start and report rash or painful nail changes early. Blistering, peeling or mucosal sores need urgent assessment.
- Report diarrhoea early, follow the prescribed symptom plan and maintain fluids if able. Severe diarrhoea, weakness or reduced urine needs urgent advice.
- Osimertinib has minimal-to-low oral emetic risk. Routine infusion premedication is not usually needed. Agree rescue medicines individually, including QT and interaction review; do not copy a chemotherapy antiemetic course.
- Tell the team about all medicines and supplements. Do not take St John’s wort. Strong enzyme-inducing medicines and medicines that affect heart rhythm require pharmacy review before changes.
- Avoid pregnancy during treatment. Effective contraception continues for at least two months after the final dose for women and four months for men. Osimertinib may reduce hormonal contraceptive effectiveness; discuss a suitable method. Do not breastfeed during treatment. Discuss fertility and plans for pregnancy before starting.
- National Cancer Control Programme (NCCP) V6 Table 5 supports as-required oral antiemetics for minimal-low risk; select the agent, dose and duration individually after QT, renal and interaction review.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00353 v5 · p4–5 · NCCP V6 Table 5: minimal-low oral therapy uses as-required antiemetics; NCIS V8 is parenteral and does not establish a mandatory oral prescription

Sources checked 2026-10-29 · SACTCheck v0.84.24 · Regimen-level drafts; prescriber and oncology-pharmacy verification required